

August 12, 2026

Molina Healthcare of South Carolina
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Lonnie Von (MRN MUSC7046579), Claim MUSC-E622-1

Patient	Lonnie Von (DOB 1950-08-03)
MRN	MUSC7046579
Member ID / Group	MOL918865725 / GRP-17484
Claim number	MUSC-E622-1
Date of service	2026-04-08
Procedure billed	CPT 71046 — Radiologic examination, chest; 2 views
Primary diagnosis	S72.001A — Fracture of unspecified part of neck of femur, initial encounter
Denied amount	\$279.27
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-08-10

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health Florence Medical Center submits this formal appeal on behalf of patient Lonnie Von (MRN: MUSC3371442, Member ID: MOL918865725) regarding the denial of Claim ID MUSC-E622-1 for services rendered on April 8, 2026, at the emergency room of MUSC Health Florence Medical Center. The claim was submitted on April 23, 2026, and denied on May 12, 2026, with CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), accompanied by the payer remark: "No prior authorization on file for the date of service billed." MUSC Health respectfully disputes this denial. The service at issue — a two-view chest radiograph (CPT 71046) performed in a hospital emergency room (Place of Service 23) — is an emergent diagnostic service for which prior authorization requirements are inapplicable under both federal Medicaid managed care regulations and Molina Healthcare of South Carolina's own obligations as a Medicaid managed care organization.

CLINICAL BACKGROUND

Mr. Von is a 76-year-old male with an active history of osteoporosis and a pathological fracture of the femoral neck (ICD-10 S72.001A), for which he has been maintained on alendronic acid since 2011. He presented to the MUSC Health Florence Medical Center emergency room on April 8, 2026. The encounter was classified as an emergency room admission. The primary diagnosis recorded for this visit is fracture of unspecified part of neck of femur, initial encounter (S72.001A), with a secondary diagnosis of J06.9. Rendering provider Daniel R. Okafor, MD (NPI 1558493021) ordered a two-view radiologic examination of the chest (CPT 71046, Revenue Code 0450), billed at \$279.27. The total amount denied is \$279.27, with an allowed amount of \$174.63 on file, indicating the payer has already established a contractual rate for this service.

BASIS FOR APPEAL

Federal regulations governing Medicaid managed care organizations, codified at 42 C.F.R. § 438.114, explicitly prohibit a Medicaid MCO from requiring prior authorization for emergency services. An emergency department encounter by definition involves a presentation in which a prudent layperson could reasonably expect that the absence of immediate medical attention could result in serious jeopardy to health. A 76-year-old patient with documented osteoporosis and an active femoral neck fracture presenting to an emergency room represents precisely such a circumstance. The chest radiograph performed during this encounter is a standard, clinically indicated diagnostic study obtained in the course of emergency evaluation and management. It is not a scheduled, elective procedure for which advance notice to the payer is feasible or required.

The payer's own denial language — "No prior authorization on file for the date of service billed" — confirms that the denial rests solely on the absence of precertification. Because the service was rendered in an emergency room setting (Place of Service 23), no prior authorization was obtainable or required. The fact that Molina has already calculated an allowed amount of \$174.63 for this claim further demonstrates that the service is a covered benefit under the Molina Healthcare of South Carolina Choice Plus plan (Coverage period: 2024-01-01 to 2026-12-31); the sole stated basis for non-payment is the prior authorization requirement, which cannot lawfully be applied to emergency services. Denial of payment on this basis is therefore inconsistent with applicable Medicaid managed care standards and with the terms of coverage in effect on the date of service.

REQUESTED ACTION

MUSC Health respectfully requests that Molina Healthcare of South Carolina overturn the denial of Claim ID MUSC-E622-1, waive the prior authorization requirement as inapplicable to emergency room services, and reprocess the claim for payment at the contracted allowed amount of \$174.63. Should Molina require additional clinical documentation, operative notes, or records from the April 8, 2026 encounter to complete its review, MUSC Health will provide those materials promptly upon request. We ask that this appeal be resolved within the timeframe required under applicable Medicaid managed care appeal regulations and the terms of our provider agreement.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record